



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : ابتسام شعبان عبدالمنعم عي
المحافظة : الغربية
الحى / المدينة : المحله اول
رقم البطاقة : 28511151602321 رقم التليفون : 01111166333
العنوان : 2 ش المهدي
العمر (تاريخ الميلاد) : 15-11-1985 المهنة : ربه منزل
تاريخ تسجيل الإستماره : 24-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
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التوقيع: -----
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أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- / د. / ----- / د. / ----- / د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
قسم : -----
جهة العلاج : -----
نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: ايتسام شعبان عبدالمنعم ع National ID: 28511151602321	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



DMS
تاكسي
تاكسي



شهادة

تشهد النقابة العامة للمحامين بأن الأستاذة / ابتسام شعبان عبدالمنعم على الشامي
المحامية

المقيدة تحت رقم / 362201

غير مشتركة بمشروع العلاج بالنقابة العامة للمحامين ولم يتم الاستفادة منه لهذا
العام 2025.

وذلك

لتقديمها لأي جهة يرغب بها دون أدنى مسئولية على النقابة العامة للمحامين .

عضو المجلس

التوقيع /

التاريخ / 2025/11/18



MDT FORM

71 / 1 / 2026 G

Name	إبتسام شعبان عبد المنعم	MRN	٢٩٤٦ / ٢٠٢٥
Age	40y	ID	٢٨٥١١١٥٦٠٢٢٢١
Menopausal status	pre	Phone	٥١٥٦١٦٤٤١٤٣
NRS SCORE		Marital status	
BMI		Family history	+ve < more breast
CO morbidity	medically free.	REASON OF MDT	post operative

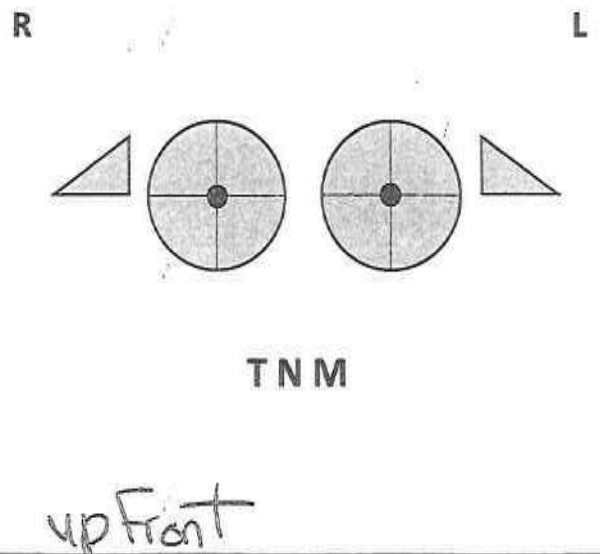
Clinical Data

BX: IDC II with intermediate DCIS

ER: 8/8 PR: 8/8 HER 2: 2+ not amplified Ki67: 5%

Sonomammography:

Mets Workup:



Recommendations: pt underwent Rt skin sparing mastectomy + SLNB then Completion axillary clearance. surgical free margins and (4/13) L.N with extracapsular extension. Stage → pT1c, pN2a. *Omnia 24/12/2025*

MDT OUTCOME & PLAN In view of high pT1c N2 (4/13 ECE) for:
① Aggressive chemo AC + T → 4 cycles x 4 ② Brachy (over hot points)
③ PORT
④ Aggressive RT (2000 cGy / 15 fractions) ⑤ 3rd cycle of chemo

⑤ BRCA test for		SIGNATURE			
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
<i>Omnia</i> 7/1/2025	<i>Omnia</i> 7/1/2025	<i>Omnia</i> 7/1/2025			

Patient Name	: الأستاذ / ايتسام شعبان عبد المنعم على	Request No.	: 13007742
Patient No.	: 28511151602321	Request Date	: 20/10/2025
Path No.	: SR-1460/25	Verification Date	: 09/11/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 39 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: marwa mohamed yous

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast mass.

Nature of Specimen: Slide revision of TCNB

Gross Examination:

-Seven slides labelled by (12826/25) were received including IPT slides (ER,PR, HER2 ,KI67).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation +nuclear pleomorphism+ mitosis): (2+2+2) Grade 2
- * DCIS: Intermediate grade , solid and cribriform with comedo subtypes
- * Lobular intra epithelial neoplasia : Absent
- * Angioinvasion: Absent .
- * Microcalcification : Positive.
- * Perineural invasion: Absent
- * IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear staining in 70% of tumor cells (score 8/8).
- PR: Strong nuclear staining in 90% of tumor cells (score 8/8).
- Her2: Moderate incomplete membrane staining in more than 10% of tumor cells (score 2+)
- SISH :
- Average HER2 copy No .2.15
- HER2/CEP17 ratio :1.04
- KI67 : 5 %

Diagnosis:

Right breast mass, Slide revision :

Invasive duct carcinoma grade 2 (tubular differentiation 3, nuclear pleomorphism 2, mitosis 2) , according to Nottingham grading system of breast cancer ,

Verified by :

Dr.Heba Hasan

Printing Date : 09/11/2025 11:17:33

Page 1 of 3

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

atient Name الأستاذ / ابتسام شعبان عبد المنعم على
atient No. 28511151602321
ath No. SR-1460/25
ex Female
ge 39 Y
effering Doc.

Request No. 13007742
Request Date 20/10/2025
Verification Date 09/11/2025
Financial Category جوستاف روسي - نفقة الدولة
Department Out Patient
Registered by .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Intermediate grade duct carcinoma insitu ,solid and cribriform with comedo subtypes with positive
Location .

ER: Positive (score 8/8).
PR: Positive (score 8/8).
Her2: Equivocal (score 2+).
SISH : Not amplified.
KI67: 5 %

luminal A.

Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1,
1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%
and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2,
intermediate; and 3, strong). Total score from 0 to 8.

Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining
observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of
invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely
perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and
within >10% of
the invasive tumor cells or complete and circumferential membrane staining that is intense and within <
10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete,
intense > 10% of tumor cells.

Signature
Prof Dr. Maha Guimei

Verified by :
Dr. Heba Hasan

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Patient Name : الأستاذ / ابتسام شعبان عبد المنعم على
Patient No. : 28511151602321
Path No. : 3697/25
Sex : Female
Age : 40 Y
Referring Doc :

Request No. : 13003691
Request Date : 29/11/2025
Verification Date : 25/12/2025
Financial Category : نفقة الدولة - جوستاف روسي
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast cancer, Up-Front.

Nature Specimen: Right skin sparing mastectomy, Sentinel lymph nodes biopsy & Completion axillary clearance.

Gross Examination:

- * Right skin sparing mastectomy specimen measured 18x17x5 cm, with overlying unremarkable nipple & areola complex measured 4x1 cm.
- * Serialling showed a mass 1.9x1.5x1 cm, located 1 cm from the deep margin & 0.7 cm from the superficial margin.
- * Surrounding breast showed rubbery grey white cut section with beaded foci.
- * SLNs: Multiple fibrofatty tissue pieces collectively measured 4x4 cm, entangling four lymph nodes, the largest measured 1.5x1x0.5 cm.
- * Completion axillary lymph nodes: Axillary fat 6x5x2 cm, entangling 9 lymph nodes, the largest measured 2x1x0.5 cm.
- * Total-nodal count is; Thirteen lymph nodes.

16 Blocks:

1- Superior margin., 2- Inferior margin., 3- Lateral margin., 4- Medial margin., 5- Deep margin., 6- Superficial margin., 7:8- Mass., 9:12- Surrounding breast., 13- SLNs., 14- N&A., 15:16- ALNs.

- Gross was done by; Dr. Heba El-Shafeey.

** Frozen (intra-operative) results:

- SLNs: Positive metastatic deposits in two out of four sentinel lymph nodes (2/4).

Microscopic Examination:

- * Histological Type: Invasive duct carcinoma.
- * Grade 2 (differentiation 2, nuclear pleomorphism 2, mitosis 2).

Verified by :



Patient Name : الأستاذ / ابتسام شعبان عبد المنعم على	Request No. : 13003691
Patient No. : 28511151602321	Request Date : 29/11/2025
Path No. : 3697/25	Verification Date : 25/12/2025
Sex : Female	Financial Category : نفقة الدولة - جوستاف روسي
Age : 40 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

- * Ductal carcinoma in situ: Extensive intermediate grade DCIS.
- * Additional findings: Surrounding breast showing foci of ductal carcinoma in situ.
- * Angioinvasion: Present.
- * Perineural invasion: Absent.
- * Surgical margins: Free.
- * Sentinel lymph nodes: Positive metastatic deposits in two out of four sentinel lymph nodes (2/4).
- * Completion axillary lymph nodes: Positive metastatic deposits in two out of nine lymph nodes (2/9), with extra-nodal extension.
- * Total nodal status: Positive metastatic deposits in four out of thirteen lymph nodes (4/13) with extra-nodal extension.

Diagnosis:

Right breast cancer (Up-front), Right skin sparing mastectomy, Sentinel lymph nodes biopsy & Completion axillary clearance:

- Invasive duct carcinoma, Grade 2, with extensive intermediate grade ductal carcinoma in situ.
- Surgical margins: Free.
- Total lymph nodal status: Positive metastatic deposits in four out of thirteen lymph nodes (4/13) with extra-nodal extension.
- Stage: pT1c, pN2a.

Signature

Prof. Dr. Gamal Abd-El-Aty.
Dr. Heba El-Shafeey.

Verified by :

Printing Date : 28/12/2025 09:26:18

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient ID: 90068	Patient Name English: Abtesam Shaban Abd Almonem Ali
Age: ٣٩ سنة, ١١ شهر	2025/11/12 12:24:21 PM :Report Date

Women's Health Outreach Program

Bilateral full field Digital Mammography

Clinical summary: 40 yrs old Right breast lump

Technique

Medio-lateral oblique and CC views & High resolution ultrasound

Detailed Findings

Right breast

- Scattered fibroglandular densities (ACR B)
Right UOQ irregular shaped faint dense spiculated lesion showing internal suspicious microcalcifications seen 6.5 cm from nipple.
- Scattered suspicious clusters of microcalcifications seen involving most of the breast quadrants .The estimated area measures 7.5 x6 cm seen about 3 cm from nipple
- Ultrasound
- Right UOQ irregular shaped spiculated hypoechoic lesion seen at 10 o'clock zone c measuring 1.8x 1.4 cm .
- Retroareolar dilated ducts .
- Normal skin and nipple/areolar complex.
- Non-specific axillary LNs.

Left breast

- Scattered fibroglandular densities (ACR B)
- No masses, asymmetric densities, distortion or suspicious clusters of microcalcifications.
- Ultrasound demonstrated no solid or cystic lesions.
- Normal skin and nipple/areolar complex.
- Non-specific axillary LNs.

Impression:

Right breast (BIRADS 6): Right UOQ pathologically proved malignant lesion associated with wide spread suspicious malignant microcalcifications

Left breast (BI-RADS 1) : Normal findings

Recommendations:

Right breast: : MRI may be advisable to delineate the extent of pathology.

Best regards,
Dr. Heba Ahmad ,MD

Patient Name: Ebtesam Shaaban Abd Al Moneim Ali	Date : 20/10/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 40 Year(s)
Medical ID: 2926/2025	Pat. ID: 132456

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS

REVEALED:

I- CHEST:

- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal size, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

- ❖ A known case of right breast cancer showed no hepatic or pulmonary metastatic lesions.

Dr. Asser Ahmed

Dr. Asser Ahmed

Name : Ibtisam Shaaban Abdel-Moneim Ali

Date : 25-10-2025 10:02

Branch : Dokki

Medical No. : 233068

Ref.Physician Prof : Gostaf Roseh Hospital

Ins.Company : تعاقد مستشفيات

Tc^{99m} MDPSKELETAL SCINTIGRAPHY

History

- A 40-year-old lady, with history of right breast cancer and referred for metastatic workup.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings

- No abnormal active or cold focal bony lesions that could account for metastatic bone disease.
- Degenerative changes are noted at the spine and big joints.

Impression:

- The skeleton is free from metastases.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine



ALRHMA
Scan

أ.د. رشا الشافعي

أستاذة بجامعة طنطا

أ.د. رشا الحظفي

أستاذة بجامعة طنطا

أ.د. محمد الشافعي

أستاذ بجامعة طنطا

أ.د. عمر حسنين

أستاذ بجامعة طنطا

Pat. Name Mrs. Ebtessam Shaaban

Pat.ID: 1010160

Abd Al Moneim Ali Al Shamy

Date: 30/08/2025

Ref.Prof.Doc:

MAMMOGRAPHY BOTH BREAST (CRANIO-CAUDAL AND MEDIO-LATERAL OBLIQUE VIEWS) FOLLOWED BY ULTRASONOGRAPHY:

- Both breasts are of fibro-fatty parenchymal pattern ACRb.
- Homogenous breast parenchyma showing normal fibro-fatty components.
- Prominent thickened upper outer quadrant fibrous parenchyma reflecting element of bilateral fibro-adenosis
- The right breast showing well defined hypoechoic focal lesion seen ar upper outer quadrant region at 9 o'clock measuring about 1.3 x 1 cm.
- No micro or macrocalcifications.
- Normal skin and subcutaneous tissue.
- No nipple retraction.
- No retroareolar duct ectasia noted.
- No suspicious axillary lymph node is noted.

OPINION:

- *Right breast mass lesion 1.3x1 cmBI-RADS 4A (FNAC is recommended).*

DR. MOHAMED AWAD(M.SC, FEBR)

٠١٠٩٦٧٨٤٤٣٣ - ٠٤٠/٢٢٢٥١٠٣ - ٠٤٠/٢٢٢٥١٠٠
Mahalla ELKobra Roundabout Club Baladiyat
Moheeb St, Bldg Sayeda zeinab

المحلة الكبرى - شارع مجيب
دوران نادي البلدية - عمارة السيدة زينب